



February 23, 2026

[Submitted electronically to <http://www.regulations.gov>]

The Honorable Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services (HHS)
Hubert H. Humphrey Building
200 Independence Ave, SW
Washington, DC 20201

RE: [\[RIN 0955-AA13\]](#) Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Secretary Kennedy,

The American Pharmacists Association (APhA) appreciates the opportunity to provide comments on CMS's "Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care," request for information. APhA writes to CMS to offer comments on behalf of our pharmacist members on artificial intelligence (AI). In 2024, APhA's House of Delegates (HOD) adopted the following resolution:

Artificial Intelligence Use in Pharmacy Practice

1. APhA opposes the replacement of a pharmacist's professional judgment or patient's access to their pharmacist with artificial intelligence.
2. APhA calls on the profession of pharmacy and all related organizations to proactively assess and respond to the evolving role of artificial intelligence in pharmacy practice and workforce dynamics.
3. APhA encourages judicious use of artificial intelligence by pharmacists and pharmacy personnel as a tool to elevate pharmacy practice and enhance patient care.

4. APhA advocates for the integration of pharmacists into the development, design, validation, implementation, and maintenance of artificial intelligence solutions.
5. APhA calls on regulatory bodies, employers, and other relevant parties to develop laws, regulations, and policies as applicable for artificial intelligence to ensure patient safety, privacy, public awareness, and public protection.
6. APhA calls on those providing artificial intelligence solutions to implement processes that identify and mitigate bias and misinformation in artificial intelligence.
7. APhA advocates for education providers to facilitate, and pharmacy personnel to seek out, education and training on the lawful, ethical, and clinical use of artificial intelligence.

APhA represents pharmacists, student pharmacists, and pharmacy technicians in all practice settings, including but not limited to community pharmacies, hospitals, long-term care facilities, specialty pharmacies, community health centers, physician offices, ambulatory clinics, managed care organizations, hospice settings, and government facilities. Our members strive to improve medication use, advance patient care, and enhance public health.

HHS's "OneHHS" AI strategy "focused on the Department's internal interests and ambitions to harness the transformative potential of AI and embed it into HHS's operations, while upholding patient privacy, civil rights, and civil liberties."¹ HHS is seeking "feedback from those building, buying, evaluating, using, and receiving care from AI tools that are part of clinical care as well as from those who wish to do so but face barriers."² HHS provides that the information it receives will be utilized to shape agency policy on the regulation, reimbursement, and research and development associated with AI.³ APhA believes there are ample opportunities for HHS, APhA, and our nation's pharmacists to work together within the context of our HOD policies to advance AI adoption in pharmacies and improve Americans' health. Accordingly, APhA provides the following comments on these three topic areas and responses to some of the questions posed by HHS below.

Regulation (FR 60109)

¹ Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care, 90 Fed. Reg. 60108, 60109. Available at: <https://www.federalregister.gov/d/2025-23641/p-12>.

² *Id.*

³ *Id.*

HHS notes the agency is the “nation’s principal health regulator” and that it “seeks to establish a regulatory posture on AI that is well understood, predictable, and proportionate to any risks presented to enable rapid innovation while protecting patients and the confidentiality of their identifiable health information, and maintaining public trust.”⁴ Given HHS’s role, APhA calls on HHS to develop AI regulations and policies that ensure patient safety, privacy, public awareness, and public protection. Regarding patient safety, AI should not replace a pharmacist’s professional judgment. AI should be used to enhance the care pharmacists provide and reduce administrative burdens that often divert them from providing direct patient care services. Regulations protecting patient privacy and their associated electronic health information should also be a top priority for HHS, as patients worry that information about their medical conditions and histories will be shared inappropriately. Health care providers and patients are also concerned that AI will provide users with biased or misleading information. Accordingly, APhA calls on HHS to implement processes to identify and mitigate bias and misinformation in the AI technologies it uses. Additionally, the public must trust that the regulations HHS and other agencies put in place will, in fact, protect their data while yielding accurate information to health care providers. As such, APhA encourages HHS to promote what the agency, the administration, and those in the private sector are doing to ensure that any AI-embedded technologies used in health care are accurate, safe, and protect patient data. Public buy-in is critical to the successful adoption of AI, and HHS is uniquely positioned to inform the public about its benefits. APhA is happy to assist HHS in sharing information about the agency’s use of AI with pharmacists, pharmacy technicians, pharmacy personnel, and patients.

Reimbursement (FR 60109)

HHS also “seek[s] to ensure that the potential promises of AI innovations are not diminished through inertia and instead such payment systems are modernized to meet the needs of a changing healthcare system.”⁵ More specifically, HHS “seek[s] feedback on payment policy changes that ensure payers have the incentive and ability to promote access to high-value AI clinical interventions, foster competition among clinical care AI tool builders, and accelerate access to and affordability of AI tools for clinical care.”⁶ AI has the potential to automate billing and claims processing. Through this automation, there is also the potential for greater transparency into the reimbursement and payment models currently in place, which could improve accuracy and reduce fraud, waste, and abuse. Additionally, by automating these processes, health care providers, including pharmacists, will have more time to focus on patient care rather than on administrative tasks and paperwork. Incentivizing those involved in reimbursement and payment processes to adopt AI for these goals will help lower overall health care spending by reducing fraudulent and inaccurate claims and improving health outcomes for Americans. APhA encourages HHS to ensure that pharmacists are involved in the

⁴ *Id.* Available at: <https://www.federalregister.gov/d/2025-23641/p-23>.

⁵ *Id.* Available at: <https://www.federalregister.gov/d/2025-23641/p-24>.

⁶ *Id.* at 60110.

rollout of such technologies and that pharmacists and pharmacies are eligible for any incentives, as they play an instrumental role in providing patient care across the country.

Research & Development (FR 60110)

HHS is also “seek[ing] input on ways in which [they] may invest in research & development (including public-private partnerships and cooperative research and development agreements (CRADAs)) to integrate AI in care delivery and create new, long-term market opportunities that improve the health and wellbeing of all Americans.”⁷ By supporting the use of certain technologies, more health care providers, including pharmacists, will feel comfortable using them in their daily practice. Additionally, by working with pharmacy schools and innovators in the AI sector, HHS can incorporate technologies in their designs to ensure AI provides accurate information, protects patient privacy, and optimizes patient care. APhA encourages HHS to seek opportunities with pharmacy schools, universities and pharmacy companies that research and develop tools for pharmacists and pharmacies. Pharmacists know where potential pain points lie in practice and can serve as a resource for these innovators, helping them identify appropriate uses of AI in both claims processing and direct patient care. As such, APhA urges HHS to consult with pharmacists who are actively using AI in practice or innovating in this space, not only at the time of rollout for one of these new technologies, but also during development and ongoing real-time improvements.

Specific Questions (FR 60110)

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

One of the top concerns APhA hears from members is uncertainty about how AI use is regulated. This creates risk, and an entity’s legal team will often determine that the risk is too high. In line with this barrier is the potential liability concerns associated with its use. APhA encourages judicious use of artificial intelligence by pharmacists and pharmacy personnel as a tool to elevate pharmacy practice and enhance patient care rather than to replace a pharmacist’s professional judgment. To ease these barriers, HHS needs to provide health care providers and entities with greater guidance regarding the use of AI-embedded technologies. HHS could also, where appropriate and without compromising patient safety or privacy, use its enforcement discretion to minimize fines or other penalties associated with potential AI-related mistakes.

Other concerns regarding the adoption of AI technologies include cost, workforce integration, and patient perception. Many health care entities, including pharmacies, are already struggling financially. Thus, adding on the costs of these technologies without knowing the true benefit is often deemed too risky for many health care entity leaders and pharmacy owners. HHS should assess ways to reimburse and incentivize pharmacists for providing patient care services as well

⁷ *Id.* Available at: <https://www.federalregister.gov/d/2025-23641/p-25>.

as fund pharmacies, hospitals, and other entities to implement such technologies. Regarding workforce integration, many health care providers, including pharmacists, lack sufficient training and education on AI, leading them to be skeptical of its use. To address this knowledge gap, HHS could provide guidance and training on the appropriate use of AI. Additionally, HHS could work with schools of pharmacy to ensure that the next generation of pharmacists is equipped with the knowledge to utilize AI-embedded technologies to optimize patient care. Regarding patient perceptions, many patients have negative views of the rise of AI in health care, fearing it could provide health care providers with inaccurate information or replace the interaction they have with them.⁸ Because of this, health care providers avoid using it, as they do not have to explain its use to their patients. As such, HHS must play an active role in educating patients and the public about the benefits of AI-embedded technologies in health care and how potential risks are mitigated to guarantee patient safety.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

As discussed above, there are many barriers to the broader adoption of AI in clinical care by both health care providers and health care entities. HHS needs to not only address these barriers but also incentivize use to support adoption. Incentivizing use could take many forms, such as expanded funding, increased reimbursement, or workforce training. HHS's early investment in adopting these technologies would be justified, as more accurate claims could reduce fraud, waste, and abuse. Additionally, it could lead to better patient health outcomes and a more satisfied health care workforce, as health care providers would spend more time using their training to provide direct patient care services rather than completing administrative tasks.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

The use of AI has yielded and will continue to raise novel legal issues related to liability, indemnification, privacy, and security.⁹ The potential of these issues alone has been enough to

⁸ Alec Tyson, et al., *60% of Americans Would Be Uncomfortable With Provider Relying on AI in Their Own Health Care*, Pew Research Center (Feb. 22, 2023). Available at: <https://www.pewresearch.org/science/2023/02/22/60-of-americans-would-be-uncomfortable-with-provider-relying-on-ai-in-their-own-health-care/>.

⁹ See Nithesh Naik, et al., *Legal and Ethical Consideration in Artificial Intelligence in Healthcare: Who Takes Responsibility?*, 9 *Frontiers in Surgery* 1 (2023). Available at: <https://pmc.ncbi.nlm.nih.gov/articles/PMC8963864/>. See also Clara Cestonaro, et al., *Defining Medical Liability When Artificial Intelligence Is Applied on Diagnostic Algorithms: A Systematic Review*, 10 *Frontiers in*

keep certain entities and health care providers from utilizing any AI-embedded technologies in their practice. As such, HHS will have to address these concerns to support greater adoption of these technologies. APhA encourages HHS to provide additional guidance and standards for the use of AI to better inform pharmacists, other health care providers, and those working in health care entities about appropriate use. HHS could also partner with professional health care associations, such as APhA, to accredit and certify AI-embedded technologies and their providers through educational training to ensure users know they are accurate and properly vetted.

To support the adoption of AI in non-clinical areas, HHS could also consider using its enforcement discretion to ease penalties for errors that do not involve patient health or patient information. For example, by recognizing the benefits of AI-embedded technologies for administrative tasks such as billing, HHS could reduce fines or penalties associated with errors in the early stages of adoption.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

As mentioned above, pharmacists, pharmacy technicians, other pharmacy personnel, other health care providers, and those working for health care entities all need to know how to use AI appropriately and which AI-embedded technologies are safe for their roles. HHS can accredit and certify AI-embedded technologies and their providers, giving health care providers and entities confidence that these technologies have been appropriately tested. As such, APhA encourages HHS to work with the private sector, such as APhA, to ensure that the products used to provide care by health care providers, such as pharmacists, and the processes associated with pharmacy and pharmacy services claims are appropriately reviewed. The more ways HHS can assure pharmacists and other health care providers of the standards for using these technologies and of how associated risks have been mitigated, the greater the trust they will have in using them.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

There are many areas where AI algorithms and technologies have been embedded into pharmacy practice to automate tasks, improve clinical decision-making, and promote patient

Medicine 1 (2023). Available at:

<https://pmc.ncbi.nlm.nih.gov/articles/PMC10711067/#:~:text=Absence%20of%20clinical%20AI%20related,AI%2Ddeveloper's%20liability%20is%20unclear.>

engagement.¹⁰ These technologies are used to assist with prescription verification, dosing of high-risk medications, processing handwritten and faxed prescriptions, adherence monitoring, and outreach to patients about refills and appointment scheduling. As the adoption of AI expands into pharmacy practice, APhA opposes replacing a pharmacist's professional judgment or a patient's access to their pharmacist with AI. APhA calls on HHS to develop regulations, standards, and policies to promote the use of AI while ensuring patients still have access to the trusted advice and care provided by their local pharmacist.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Federal agencies, including HHS, should work with state boards of pharmacy to ensure that AI is properly integrated into pharmacies and into the care pharmacists provide. Boards of pharmacy play a significant role in outlining the scope of practice for pharmacists and pharmacy technicians, as well as the corresponding legal boundaries, which could affect the use of AI-embedded technologies by those working in pharmacies. Boards of pharmacy also often license these individuals and may require continuing education to maintain pharmacist licensure. HHS should partner with state boards of pharmacy to ensure that all pharmacists and pharmacy technicians receive training on AI-embedded technologies, their appropriate uses, and potential risks. APhA encourages HHS to consider the interplay between potential AI regulations and state scope-of-practice laws to ensure that pharmacists can judiciously use AI in their practice while complying with all applicable state and federal laws and regulations.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

By enhancing interoperability, pharmacists, pharmacy technicians, and pharmacy personnel will have greater access to the patient's electronic health record. This not only provides pharmacists with additional medical information to deliver better patient care but also gives other health care providers greater insight into the patient's medication history. One study found that patients reported 43% of medications in their electronic health records were inaccurate, with 29% discontinued and 14% changed.¹¹ Greater access to these records will help

¹⁰ Craig Kimble, et al., *Intelligent Pharmacy Leveraging AI and Automation to Enhance Patient Care and Pharmacist Roles*, 14 *Pharmacy Times* 1 (Sept. 19, 2025). Available at: <https://www.pharmacytimes.com/view/intelligent-pharmacy-leveraging-ai-and-automation-to-enhance-patient-care-and-pharmacist-roles#:~:text=AI%20and%20Automation%20in%20Today's,errors%20and%20enabling%20faster%20turnaround..>

¹¹ Maria Staroselsky, et al., *An Effort to Improve Electronic Health Record Medication List Accuracy Between Visits: Patients' and Physicians' Response*, 77 *International Journal of Medical Informatics* 153 (2008).

both prescribers and pharmacists avoid duplicate therapies, address adherence concerns, minimize adverse reactions, and ensure the medications a patient is taking correspond to an appropriate condition or disease state. APhA supports efforts to enhance interoperability, allowing pharmacists to access electronic health information from other health care providers and to share patient health information collected at the pharmacy with them. As a reminder, APhA [commented](#) on ONC's proposed rule, "[Health Data, Technology, and Interoperability: Patient Engagement, Information Sharing, and Public Health Interoperability](#)." Within this proposed rule, ONC moved to incorporate the Public Health Service Act (PHSA) section 3000 definition of "pharmacist" into the definition of "health care provider" for information-blocking purposes. APhA "commend[ed] ONC for taking steps to enhance the interoperability of EHI amongst health care providers" and "support[ed] pharmacists being recognized as providers in relation to information-blocking regulations discussed in the proposed rule as long as pharmacists also have access to EHI to ensure the delivery of pharmacist-provided patient care services [emphasis added]." APhA also requested ONC to require other health care providers and developers of health IT certified under the final rules to be prohibited from information blocking EHI to pharmacists and penalize developers for doing so. APhA reiterates these points as it is urgent that pharmacists gain real-time access to essential electronic health information of their patients because information blocking causes delays in patient care and prevents health care providers from having a complete picture of the patient's health, which can lead to inefficient and ineffective care.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers are concerned that greater adoption of AI will reduce their interactions with trusted pharmacists and other health care providers.¹² Pharmacists are consistently rated among the most trusted professionals, and millions of patients rely on their expertise each year to manage their conditions.¹³ Given the accessibility of pharmacies, pharmacists are often viewed as the first line of care for patients with minor ailments and conditions, especially in rural and underserved areas, where pharmacists may be the only readily available health care provider. As such, APhA urges HHS to ensure that the adoption of AI does not interfere with a patient's access to their pharmacist. Ideally, the adoption of AI enables pharmacists to provide

Available at:

<https://www.sciencedirect.com/science/article/abs/pii/S1386505607000470#:~:text=This%20study%20confirms%20earlier%20reports,and%2014%25%20having%20been%20changed.>

¹² Alec Tyson, et al., *60% of Americans Would Be Uncomfortable With Provider Relying on AI in Their Own Health Care*, Pew Research Center (Feb. 22, 2023). Available at:

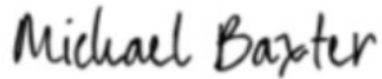
<https://www.pewresearch.org/science/2023/02/22/60-of-americans-would-be-uncomfortable-with-provider-relying-on-ai-in-their-own-health-care/>.

¹³ Megan Brennan, *Nurses Continue to Lead in Honesty and Ethics Ratings*, Gallup (Jan. 12, 2026). Available at: <https://news.gallup.com/poll/700736/nurses-continue-lead-honesty-ethics-ratings.aspx>.

more direct patient care without compromising patient safety or the quality of their interactions with patients.

Thank you for the opportunity to submit comments on this notice. If you have any questions or would like to meet with APhA and our nation's pharmacists, please contact Corey Whetzel, APhA's Senior Manager, Regulatory Affairs, at cwhetzel@aphanet.org.

Sincerely,

A handwritten signature in black ink that reads "Michael Baxter". The script is cursive and fluid, with the first letters of each word being capitalized and slightly larger than the rest of the letters.

Michael Baxter
Vice President, Government Affairs